

GROWTH CONCEPT

METSI Care

**Small physician-owned practices.
A shared platform for growth.**

Proposed operating model and investment logic

Prepared by Drew Cleaver / September 2026

Independent concept for discussion. No approved METSI partnership, new offer or investment solicitation.

A trusted local doctor is the product. A repeatable practice is the growth engine.

Patient proposition

A familiar physician, predictable monthly pricing and a clear plan after each visit.

Practice design

One owner and up to two associates.
Protected time for follow-up and urgent needs.

Expansion mechanism

Shared nonclinical operations and a mentored path from associate to practice owner.

Investment condition

Prove retention and margin after fair physician pay before committing to a network.

Two visit lengths make the monthly relationship easier to schedule

Proposed terms	Core	Extended
Monthly dues	\$200	\$300
Included monthly visit	20-25 minutes	45 minutes
Calendar block	30 minutes	60 minutes
Optional annual prepay, 20% off	\$1,920 / year	\$2,880 / year

Both: remote access within published coverage. Extra visits: \$60 up to 25 minutes or \$100 around 50 minutes. Discounted labs cost extra. Membership is not insurance.

Each encounter ends with an agreed plan and a clear route back to care

Moment	Patient experience
Before the visit	Brief agenda, medication updates and relevant results
During the visit	Prioritize concerns, address risk and agree on next steps
After the visit	Physician-reviewed summary, owners and follow-up dates
Between visits	Routine reply target by next business day during coverage
Urgent needs	Clinical triage route and held slots, with backup coverage

Monthly visits remain optional and based on need. No rollover. Emergency care and specialist psychotherapy sit outside this membership.

Illustrative proposal. All operating assumptions require validation.

Six scheduled visits preserve time for access and clinical follow-through

Owner schedule: Tuesday-Thursday	Time allocation
10:00 am–1:00 pm	Three visit blocks
1:00–2:00 pm	Lunch, reset and case review
2:00–5:00 pm	Three visit blocks
5:00–6:00 pm	Held urgent capacity and same-day needs
6:00–7:00 pm	Results, messages and closeout

Core visits use 30-minute blocks. Recovered time supports care first. Associates cover five weekdays, with fair, explicit after-hours rotations.

Illustrative proposal. All operating assumptions require validation.

A three-physician location supports 299 routine visits per month on average

Clinician capacity	Days / week	Routine visits / month
Owner physician	3	69
Two associate physicians	10 total	230 total
Location total	13	299

360 members × 70% monthly attendance = 252 visits

84% of routine slots used. At 80% attendance, utilization reaches 96%. At 90%, demand exceeds capacity. Remote workload needs its own limit.

Assumptions: 6 visits/day, 46 working weeks/year. Owner 3 days/week, two associates 5 each. Rounded outputs.

The six-visit model can cover costs, but its initial margin is thin

Annual economics	Amount
Membership revenue	\$1,188,000
Physician compensation + burden	(\$840,000)
Local nonphysician operating costs	(\$210,000)
Shared-platform fee	(\$60,000)
Operating surplus before D&A, interest, tax	\$78,000 / 6.6%

360
members at maturity

\$275
blended monthly dues
75% Extended / 25% Core
No annual discount

Break-even: 337 members at \$275, before startup costs and working capital.

Illustrative steady state, USD/year. No actual METSI financials supplied. Owner compensation is an expense.

Annual discounts can consume most of the location's operating surplus

Scenario	Net dues / month	Annual surplus	Margin
75% Extended, monthly pay	\$275.00	\$78,000	6.6%
25% prepay at 20% discount	\$261.25	\$18,600	1.6%
Everyone prepays at 20% off	\$220.00	(\$159,600)	(16.8%)
50% Extended, monthly pay	\$250.00	(\$30,000)	(2.8%)
100% Extended, monthly pay	\$300.00	\$186,000	14.4%

Prepayment improves cash timing, not earned revenue. The 20% option should remain a test, with clear cancellation and refund terms.

Each scenario uses 360 members and \$1.110m annual operating costs. No add-on, laboratory or drug margin assumed.

Austin is a focused first market for validating willingness to pay

Initial audience

Adults roughly 35–55 who value an ongoing primary-care relationship and proactive follow-up.

Offer to test

Mental-health support within primary care, evidence-based weight care and preventive planning.

Acquisition channels

Opt-in patient referrals and local professional communities, followed by small paid tests.

Evidence to collect

Paid conversion, acquisition cost, retained margin and reasons for joining or leaving.

A residency-trained associate can learn the practice before owning a location

Stage	Role and gate
Join	Licensed, residency-trained MD/DO with competitive pay
Develop	Clinical autonomy, mentoring and practice-management skills
Qualify	Quality, patient continuity and operating readiness review
Own	Potential buy-in or new site after roughly 18-36 months
Replenish	Recruit a replacement before an associate or panel moves

Maximum three physicians per practice: one owner and two associates. Business mentorship is separate from accredited medical training.

Training source: AAFP. Ownership timing and financing below are hypotheses, not commitments.

Local physicians control care.

A shared company supports operations.

Physician-owned practice	Shared nonclinical platform
Diagnosis, prescribing and medical judgment	Brand, scheduling and operating playbook
Clinical staffing and supervision	Administrative support and recruitment services
Patient relationships and clinical records	Approved systems and vendor coordination
Care quality and escalation decisions	Service reporting and practice-owner training

Illustrative platform fee: \$5,000 per location per month for defined services.
Fair-market-value support and compliant agreements require review.

Structure is conceptual. Texas healthcare and franchise counsel must review ownership, fees and contracts.

Platform fees create recurring revenue, but clinic sales are not platform sales

Mature locations	System clinic revenue / year	Platform revenue / year
1	\$1.188 million	\$60,000
25	\$29.7 million	\$1.5 million
100	\$118.8 million	\$6.0 million

The platform must cover central staffing and support from its own fees.

Conventional VC fit remains conditional on repeatable openings, retention and profitable support delivery. Patient continuity and physician trust are the potential advantage.

Illustrative footprint arithmetic at \$1.188m clinic revenue and \$60k platform fees per mature site. Not a forecast.

A 90-day pilot should precede new staffing or a franchise program

Stage	Evidence required before the next commitment
Weeks 1-2: baseline	Dr. Garrick approval, costs, time study and contract review
Weeks 3-12: 20-30 opt-in members	Paid demand, completed care plans and measured message load
Before hiring an associate	Documented demand, compensation quotes and funded runway
Before a second location	At least 6 months of stable retention, access and unit economics

Proposed thresholds: $\geq 90\%$ 90-day retention, $\geq 90\%$ routine replies within target, $\geq 90\%$ visits with an agreed plan, $\leq 85\%$ routine slot use.

The next investment is evidence that this model works without its founder

Immediate decision

Agree a limited pilot with Dr. Garrick. Preserve existing member terms and track care quality alongside revenue.

Clinical and access safeguards

Publish coverage and emergency boundaries. Fund backup care. Use appropriate referrals, testing and treatments.

Before outside capital

Target $\geq 15\%$ location operating margin after fair physician pay and platform fees. The current base case falls short.

Legal and data safeguards

Review membership, HSA, employment and franchise rules. AI notes require privacy controls and physician sign-off.

The \$1.110m cost base includes owner pay and a platform service fee

Annual expense	Assumption
Owner physician cash compensation	\$180,000
Two associates: \$260,000 each	\$520,000
Payroll burden, benefits and malpractice	\$140,000
Local support staff, fully loaded	\$90,000
Occupancy and utilities	\$60,000
Clinical technology and supplies	\$18,000
Local marketing	\$18,000
Other administration and business insurance	\$24,000
Shared nonclinical platform fee	\$60,000
Total	\$1,110,000

Planning assumptions, not quotes. Excludes startup capex, deposits, debt service, tax and platform company overhead.

Public facts and planning assumptions remain separate

1. METSI Care	Published memberships and physician background
2. AAFP	Direct primary care model and compensation context
3. AAFP	Family medicine residency and training requirements
4. Texas Medical Association	Corporate practice of medicine framework, counsel review required
5. Federal Trade Commission	Franchise disclosure and financial performance representations
6. U.S. Department of Health and Human Services	HIPAA cloud services guidance

Model: $13 \text{ clinician-days} \times 6 \text{ visits} \times 46 \text{ weeks} \div 12 = 299 \text{ visits/month}$.
Revenue = members $\times$ net monthly dues $\times$ 12. Surplus = revenue – stated annual costs.
No verified patient census, retention, CAC, lab savings or therapy-price comparison enters the model.

Sources accessed September 19, 2026. This is an independent concept, not an approved offering or an FDD.